

Master 04 — ICB Governance & Business Case

Audience: ICB governance, commissioner sponsor, DPO / IG, safeguarding, clinical governance, procurement / finance readers.

Purpose: Show how the First-50 intervention test can be governed safely before any build, participant onboarding or external claim.

Status: DRAFT — needs Dean review.

Priority: P1 / P2.

Review needed from: Governance lead; DPO / IG; safeguarding lead; commissioner sponsor; Dean.

Source basis: Master 04 source text, Programme Plan governance sections, Programme Lock, START update.

A4 REVIEW PAGE 1 — Governance summary

The first OTOS Continuity™ test should be governed as a bounded intervention test of the OTOS Continuity Management Layer and user Continuity Thread, not as a generic digital coordination project.

The governance purpose is to answer one question safely:

Can a suspected ADHD-driven addiction cohort be identified, clinically routed, supported through authorised partner activity in the Continuity Management Layer, followed through a whole-person Continuity Thread, and evidenced locally without OTOS taking clinical responsibility?

The test should not proceed until the programme has agreed:

- cohort criteria;
- partner roles;
- clinical boundary;
- safeguarding thresholds;
- information-governance route;

- consent and withdrawal process;
- SMS template rules;
- human-review workflow;
- evaluation framework;
- budget / funding route;
- stop conditions.

The programme must be capable of stopping. That is part of its credibility.

A4 REVIEW PAGE 2 — Clinical and service boundary

The Continuity Management Layer surfaces agreed continuity signals. Partner teams make decisions.

OTOS may show that a person missed a check-in, replied to an SMS, did not complete a handoff, attended a support route, went silent or requested contact. OTOS does not decide what that means clinically.

Partner services retain responsibility for:

- ADHD assessment;
- ADHD diagnosis;
- medication decisions;
- medication review;
- prescribing;
- addiction-treatment planning;
- therapy;
- safeguarding;
- crisis response;
- clinical documentation;
- eligibility decisions;
- clinical risk assessment.

OTOS does not diagnose ADHD or NIA. It does not prescribe. It does not manage medication. It does not treat addiction. It does not deliver therapy. It

does not provide crisis care. It does not make safeguarding decisions. It does not write into clinical records. It does not make automated clinical decisions.

A4 REVIEW PAGE 3 — IG, data and continuity-infrastructure governance

The first intervention test should be data-minimised, tokenised and non-clinical by design.

Core principles:

- tokenised IDs by default;
- minimum necessary contact data for SMS;
- no NHS number unless governance explicitly requires it;
- no clinical notes;
- no therapy notes;
- no prescribing instructions;
- no diagnostic records;
- no EPR write-back;
- UK hosting;
- role-based access;
- append-only audit trail;
- retention schedule agreed before launch;
- withdrawal process clear to participants.

Layer = the non-clinical infrastructure partners and the user access. Thread = the user's whole-person journey over time, built from agreed non-clinical events. Service lines = each partner's own records, communication channels, service work and responsibilities.

The Continuity Management Layer is shared continuity infrastructure, not a shared clinical record.

Where consent, role and governance allow, the user's Continuity Thread may record agreed non-clinical continuity signals and evidence-export fields. It must not become a parallel clinical record or an unofficial EPR.

Controller / processor positions must be confirmed before launch. They must not be assumed in copy or in technical design.

A4 REVIEW PAGE 4 — Safeguarding and human review

OTOS is not a safeguarding authority or crisis provider. It must operate inside partner safeguarding frameworks.

Before launch, the programme needs:

- safeguarding protocol;
- crisis-language SMS wording;
- escalation owner map;
- review-time expectations;
- out-of-hours position;
- incident-reporting route;
- participant information about emergency support;
- staff training notes.

Every agreed drift or concern signal must be routed to a human-review process.

Human review should record:

- reviewer;
- time received;
- time reviewed;
- signal type;
- decision category;
- action owner;
- action taken / no action;
- closure status;
- escalation route if required.

The system must not automate clinical triage. The purpose of the signal is to make silence visible, not to replace judgement.

A4 REVIEW PAGE 5 — Business case and gates

The current budget posture is a circa £150k planning envelope, subject to requote and governance.

This should be described as a build / evidence / governance planning envelope, not a final quote and not a rollout budget.

The business case is not "OTOS will save money."

The business case is:

The local system may already be spending late on a suspected ADHD-driven addiction cohort. A bounded First-50 test can replace assumptions with decision-grade local evidence.

Decision gates

Gate	Timing	Decision	Minimum evidence needed
Gate 0	End Phase 0	Start build or stop	Partner roles, governance, IG route, budget, evaluation plan
Gate 1	End mobilisation	Start participant onboarding	Tested workflows, SMS, review queue, staff access
Gate 2	Week 4	Continue active running or change scope	Recruitment viability, consent, early burden, safety
Gate 3	Week 8	Continue to close or narrow	Engagement, drift, review queue, unresolved risk
Gate 4	Week 12	Continue / continue with changes / rebuild / pause / stop	Evidence pack and partner feedback
Gate 5	Post Phase 1	Design next phase or stop	Commissioner and partner decision

Review notes

Dean decision needed

- ☒ ~~Confirm whether Master 04 should be an external ICB appendix or internal governance briefing first.~~

Dean note: Use as an internal governance briefing first. It can become an external ICB appendix only after governance, IG / DPO, safeguarding and clinical-boundary review.

- ☒ ~~Confirm budget wording and whether £150k remains included.~~

Dean note: Keep the circa £150k planning envelope if useful, but only as subject to requote and governance. Do not present it as a final quote, rollout budget or confirmed supplier price.

- ☐ Confirm who the intended commissioner sponsor / host route is, if known.

Evidence / citation check needed

- ☒ ~~Check any national or local figures before external circulation.~~

Dean note: Tick in principle. All national and local figures need pre-flight checks before external circulation. Source-backed figures and OTOS modelled figures must be labelled separately.

- ☒ ~~Check START wording if included in final governance annex.~~

Dean note: Tick in principle. START can stay as evidence rationale if needed, but final copy must use the exact citation and Master 09 caveats. Do not present START as medication-only proof, an OTOS outcome or a predicted 61.3% result.

Clinical / governance check needed

- ☐ Full governance review required before external use.

Dean note: Leave unticked for now. This needs governance-team input before approval.

- ☐ IG / DPO review required.

Dean note: Leave unticked for now. This needs IG / DPO input before approval.

- ☐ Safeguarding review required.

Dean note: Leave unticked for now. This needs safeguarding input before approval.

☐ Clinical boundary review required.

Dean note: Leave unticked for now. This needs clinical-team input before approval.

Copy approval

☐ Approved

☐ Needs edit

☐ Hold
